

MediLink

Normal Events Handbook

Revised Edition: Distinctive Competition Formats, Student Procedures, and Scoring Guides

20 regular events • High-school division • In-person format • Draft competition framework

Design purpose. This revised handbook is built to ensure that each regular MediLink event feels like a different professional world. Events are not distinguished only by topic; each has a unique student role, primary action, signature mechanic, deliverable, judging environment, and live pressure point.

Core standard. A student who enters multiple events should not feel that they repeated the same “read a case and give a presentation” experience. Instead, they should audit, prioritize, trace, investigate, deliberate, debate, design, plan, trade, negotiate, allocate, optimize, diligence, adjudicate, rescue, validate, found, implement, and communicate.

Competition Design Standard

Every regular event must satisfy six distinctiveness requirements. Event directors may adjust annual scenarios, but they should not remove the core mechanic that makes an event recognizable and non-duplicative.

Requirement	Meaning
1. Unique role	Students enter a different professional setting: e.g., outbreak unit, investment desk, ethics committee, operating team, or press room.
2. Unique action verb	Each event's central task is different: prioritize, audit, trace, investigate, peer-review, deliberate, debate, design, plan, trade, negotiate, allocate, optimize, diligence, adjudicate, rescue, validate, found, implement, or communicate.
3. Unique signature mechanic	Every event includes a defining live feature such as a shock card, negotiation, evidence wall, token board, incident drill, or press conference.
4. Unique work product	Events require different outputs: action board, audit worksheet, causal map, ruling, term sheet, allocation board, dashboard, communications plan, and more.
5. Unique judging environment	Judges act as appropriate stakeholders—board, editor, patient advocate, founder, investor, payer, regulator, or journalist.
6. Unique live pressure	Students must respond to an event-specific challenge rather than simply deliver a prepared speech.

This approach maintains MediLink's medicine–finance identity while creating entry points for clinical thinkers, analysts, investigators, speakers, designers, builders, entrepreneurs, policy advocates, and leaders.

Official Event Classes

Class	Before competition	At competition
On-site: Standard	Students study the published domain, skills, and sample format.	The complete prompt is released at the site; event-specific work occurs under timed supervision.
On-site: Advanced Release	Students study the published domain in advance.	The full confidential packet releases two hours before the judged round in a supervised preparation room.
Preparation-based	Students receive the specific case/prompt and submission requirements in advance.	They present, negotiate, demonstrate, defend, or respond to a live twist in person.

Integrity and supervised preparation

Unless an annual guide expressly permits a specified use, personal devices, internet access, external communication, generative AI, and outside assistance are prohibited during supervised on-site preparation. Compliance is supported through monitored rooms, device procedures, submitted work products, role documentation, and individual oral questioning. Automated AI-detector output alone should never determine a penalty.

Universal scoring principle

Every event uses a 100-point rubric. High-scoring work is accurate, case-specific, feasible, ethically aware, transparent about uncertainty, and resilient under questioning. Judges score substance and reasoning over aesthetic polish. A material safety error, fabricated evidence, prohibited assistance, or failure to meet required deliverables may result in deductions or formal review under the event rules.

Event Identity Map

#	Event	Role	Action	Signature mechanic	Basis
1	Triage Protocol	Emergency clinical-prioritization team	Prioritize	Evolving patient-status board	Advanced Release
2	The Chart Room	Patient-safety chart auditor	Audit	Line-by-line error hunt	Standard
3	System Failure	Physiology systems analyst	Trace	Causal-cascade mapping board	Standard
4	Patient Zero	Outbreak investigation unit	Investigate	Physical evidence wall and outbreak map	Advanced Release
5	Under Review	Biomedical peer-review committee	Peer-review	Confidential journal decision hearing	Advanced Release
6	The Gray Area	Clinical ethics committee	Deliberate	Stakeholder testimony and written ruling	Standard
7	The Floor	Legislative/policy advocacy team	Debate	Formal hearing with cross-examination and amendment round	Advanced Release
8	Pitch Day	Healthcare solution designer	Design	Demonstration, prototype, workflow, or storyboard requirement	Preparation
9	The Ledger	Household financial planner	Plan	Budget board with unexpected expense cards	Standard
10	Market Call	Investment desk analyst	Trade	Live market-shock ticker	Standard
11	The Term Sheet	Deal team: investor, lender, or corporate finance adviser	Negotiate	Live founder–investor/lender negotiation	Preparation
12	Scarcity	Healthcare resource-allocation analyst	Allocate	Finite-budget token board	Standard
13	Operating Margin	Service-line or clinic optimization team	Optimize	Operations dashboard and constraint round	Preparation
14	The Pipeline	Biotech/pharma due-diligence committee	Diligence	Investment committee with late clinical or patent update	Preparation
15	Claim Denied	Claims appeal and adjudication panel	Adjudicate	Two-chair payer–patient hearing	Standard
16	The Turnaround	Executive leadership team	Rescue	Multi-round boardroom rescue simulation	Preparation
17	Signal vs. Noise	Clinical technology review board	Validate	Technology approval gate and incident drill	Standard
18	Seed Round	Healthcare startup founding team	Found	Investor due-diligence gauntlet	Preparation
19	Borders & Budgets	Contextual health-implementation team	Implement	Geographic implementation board and logistics shock	Preparation
20	On Record	Health crisis communications team	Communicate	Live press room and misinformation escalation	Preparation

1. Triage Protocol

CLINICAL | Solo or team, 1–5 | ON-SITE: ADVANCED RELEASE

Student role: Emergency clinical-prioritization team **Primary action:** Prioritize **Signature mechanic:** Evolving patient-status board

Students receive a fictional acute-care scenario with symptoms, vitals, laboratory or imaging findings, operational constraints, and progressively released updates. Their task is to establish the safest order of action—not merely name a diagnosis—and explain why each step occurs now, later, or not at all.

Preparation Before Competition

- Study clinical reasoning, triage frameworks, patient safety, escalation, and interpretation of basic fictional diagnostic data. The broad annual domain is announced early; the complete case is released in a supervised room two hours before the judged round.

Competition Experience

- Teams build a one-page action board. During the round, staff release two or three new updates—such as worsening vital signs, a medication allergy, bed unavailability, or transport delay. Teams must visibly reorder priorities and defend the change.

Required Work Product

- Prioritized action board; evidence/risk map; escalation plan; oral defense.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Safety and prioritization	35	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Evidence interpretation	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Adaptation to live changes	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Feasibility and escalation	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Individual oral defense	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout Triage Protocol entry does more than answer the prompt. It fully inhabits the role of the emergency clinical-prioritization team, performs the required action—**prioritize**—and uses the signature mechanic, **evolving patient-status board**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.
- All scenarios are fictional and educational. Competitors must not present themselves as licensed clinicians or give real-person medical advice.

2. The Chart Room

CLINICAL | Solo-only | ON-SITE: STANDARD

Student role: Patient-safety chart auditor **Primary action:** Audit **Signature mechanic:** Line-by-line error hunt

Competitors act as documentation and patient-safety auditors. A fictional chart contains material inconsistencies, omissions, unsafe abbreviations, medication/allergy conflicts, date errors, or documentation weaknesses that could affect care, billing, privacy, or continuity.

Preparation Before Competition

- Study medical terminology, chart structures, safe documentation practices, patient-safety concepts, privacy, and basic billing/reimbursement vocabulary. The chart is confidential until the event.

Competition Experience

- Students complete a timed, line-referenced audit: every claimed issue must identify the exact chart evidence, likely consequence, and appropriate correction. Finalists may defend one disputed finding in a short interview.

Required Work Product

- Completed audit worksheet; ranked issue list; consequence-and-correction rationale.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Accuracy of material findings	40	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Patient-safety impact	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Documentation/terminology reasoning	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Billing, privacy, or continuity analysis	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Clarity and completeness	5	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout The Chart Room entry does more than answer the prompt. It fully inhabits the role of the patient-safety chart auditor, performs the required action—**audit**—and uses the signature mechanic, **line-by-line error hunt**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.
- All scenarios are fictional and educational. Competitors must not present themselves as licensed clinicians or give real-person medical advice.

3. System Failure

CLINICAL | Solo or team, 1–5 | ON-SITE: STANDARD

Student role: Physiology systems analyst **Primary action:** Trace **Signature mechanic:** Causal-cascade mapping board

A fictional physiologic system breaks down. Competitors trace the cascade from initiating event to compensatory changes, organ-level effects, clinical deterioration, intervention priorities, and care-resource impact. The event is a systems-mapping challenge, not a rapid triage contest.

Preparation Before Competition

- Study anatomy, physiology, pathophysiology, disease pathways, and basic implications of complications for beds, testing, staffing, and cost. The annual system focus may be announced; the case is released on-site.

Competition Experience

- Competitors construct a visual cascade map with arrows, decision points, and clinical consequences. Judges interrupt at selected links and ask the team to explain what evidence supports that connection and what would break the cascade.

Required Work Product

- Causal map; intervention nodes; clinical and operations impact summary; oral explanation.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Physiologic accuracy and causal logic	35	Specific, evidence-based, feasible, and responsive to the event’s unique role, live mechanic, and case constraints.
Depth of cascade analysis	25	Specific, evidence-based, feasible, and responsive to the event’s unique role, live mechanic, and case constraints.
Intervention reasoning	20	Specific, evidence-based, feasible, and responsive to the event’s unique role, live mechanic, and case constraints.
Healthcare-resource connection	10	Specific, evidence-based, feasible, and responsive to the event’s unique role, live mechanic, and case constraints.
Visual clarity and oral defense	10	Specific, evidence-based, feasible, and responsive to the event’s unique role, live mechanic, and case constraints.

Student Success Standard

A standout System Failure entry does more than answer the prompt. It fully inhabits the role of the physiology systems analyst, performs the required action—**trace**—and uses the signature mechanic, **causal-cascade mapping board**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.
- All scenarios are fictional and educational. Competitors must not present themselves as licensed clinicians or give real-person medical advice.

4. Patient Zero

CLINICAL | Solo or team, 1–5 | ON-SITE: ADVANCED RELEASE

Student role: Outbreak investigation unit **Primary action:** Investigate **Signature mechanic:** Physical evidence wall and outbreak map

Competitors investigate a fictional outbreak through case records, timelines, maps, exposure histories, laboratory data, and population statistics. They determine the most plausible source and transmission pathway, then propose a proportional response under resource and trust constraints.

Preparation Before Competition

- Study epidemiology, outbreak methods, epidemic curves, attack rates, surveillance, communication, and response planning. The full outbreak packet releases two hours before the round.

Competition Experience

- Teams assemble an evidence wall connecting people, places, exposures, and time. They issue a command briefing, then receive a new lab result or exposure record that may require the hypothesis to change.

Required Work Product

- Source/transmission hypothesis; evidence wall or map; response plan; communications and resource summary.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Use of epidemiologic evidence	30	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Source and transmission reasoning	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Containment and implementation plan	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Adaptation to new evidence	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Equity, ethics, and public trust	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Communication quality	5	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout Patient Zero entry does more than answer the prompt. It fully inhabits the role of the outbreak investigation unit, performs the required action—**investigate**—and uses the signature mechanic, **physical evidence wall and outbreak map**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.
- All scenarios are fictional and educational. Competitors must not present themselves as licensed clinicians or give real-person medical advice.

5. Under Review

CLINICAL | Team-only, 2–5 | ON-SITE: ADVANCED RELEASE

Student role: Biomedical peer-review committee **Primary action:** Peer-review **Signature mechanic:** Confidential journal decision hearing

Teams receive a fictional biomedical paper, trial report, or evidence dossier with a consequential methodological, statistical, ethical, reporting, or interpretation problem. They decide whether the work should be accepted, revised, rejected, replicated, restricted, or deferred.

Preparation Before Competition

- Study study design, bias, basic statistics, evidence hierarchies, clinical research ethics, conflicts of interest, and scientific communication. The complete paper packet is released two hours before the hearing.

Competition Experience

- Teams prepare a structured peer-review memorandum and issue a formal committee ruling. Judges conduct a journal-editor hearing in which each member must defend a particular claim, limitation, or recommendation.

Required Work Product

- Peer-review memorandum; decision statement; evidence audit; proposed corrective or next-study plan.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Identification of consequential flaw(s)	35	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Methodological/statistical reasoning	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Ethics and integrity analysis	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Quality of ruling and next steps	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Committee defense and role mastery	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout Under Review entry does more than answer the prompt. It fully inhabits the role of the biomedical peer-review committee, performs the required action—**peer-review**—and uses the signature mechanic, **confidential journal decision hearing**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.
- All scenarios are fictional and educational. Competitors must not present themselves as licensed clinicians or give real-person medical advice.

6. The Gray Area

CLINICAL | Solo or team, 1–5 | ON-SITE: STANDARD

Student role: Clinical ethics committee **Primary action:** Deliberate **Signature mechanic:** Stakeholder testimony and written ruling

Competitors receive a difficult healthcare dilemma involving consent, privacy, scarce resources, end-of-life care, AI, disability rights, family conflict, or equitable access. They must reach a concrete recommendation, openly identify the harm it may cause, and establish safeguards.

Preparation Before Competition

- Study ethical frameworks, consent, privacy, equity, institutional decision-making, and respectful healthcare communication. The scenario is released at the event.

Competition Experience

- After individual/team analysis, competitors hear short fictional stakeholder statements. They issue a written ruling and defend it before an ethics-chair panel. The event does not use assigned opposing sides; it tests deliberation and accountable decision-making.

Required Work Product

- Decision/ruling; stakeholder-impact matrix; safeguards and communication plan; oral defense.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Ethical reasoning and stakeholder analysis	35	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Defensibility of the recommendation	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Equity and unintended-effects analysis	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Safeguards and practical implementation	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Communication and response	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout The Gray Area entry does more than answer the prompt. It fully inhabits the role of the clinical ethics committee, performs the required action—**deliberate**—and uses the signature mechanic, **stakeholder testimony and written ruling**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.
- All scenarios are fictional and educational. Competitors must not present themselves as licensed clinicians or give real-person medical advice.

7. The Floor

SYSTEMS | Team-only, 2–5 | ON-SITE: ADVANCED RELEASE

Student role: Legislative/policy advocacy team **Primary action:** Debate **Signature mechanic:** Formal hearing with cross-examination and amendment round

Teams argue assigned affirmative or negative positions on a healthcare-policy resolution. They must show how the proposal would work, who benefits or bears risk, what it costs, and how implementation would occur. This is adversarial policy advocacy—not an ethics committee discussion.

Preparation Before Competition

- Study the announced policy domain, health-system financing, access, implementation, evidence use, and rebuttal. The official resolution, assigned side, evidence packet, and amendment details release two hours before competition.

Competition Experience

- The event includes opening advocacy, evidence presentation, cross-examination, rebuttal, an amendment or fiscal-impact round, and closing. Every member must hold a visible role such as lead advocate, evidence analyst, cross-examiner, or closing strategist.

Required Work Product

- Evidence outline; formal debate performance; amendment response; closing recommendation.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Policy and evidence analysis	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Medical/access/equity impact	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Financial and implementation analysis	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Rebuttal and cross-examination	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Team role execution	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Professionalism	5	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout The Floor entry does more than answer the prompt. It fully inhabits the role of the legislative/policy advocacy team, performs the required action—**debate**—and uses the signature mechanic, **formal hearing with cross-examination and amendment round**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.

8. Pitch Day

SYSTEMS | Solo or team, 1–5 | PREPARATION-BASED

Student role: Healthcare solution designer **Primary action:** Design **Signature mechanic:** Demonstration, prototype, workflow, or storyboard requirement

Competitors develop a solution for deployment inside an existing hospital, clinic, payer, nonprofit, school, or public-health program. Entries may be workflows, services, apps, devices, navigation systems, quality-improvement tools, or interventions—but must demonstrate a real operational path, not just an idea.

Preparation Before Competition

- The problem area and submission rules are released before competition. Students research need, build a solution, test or refine it where appropriate, and prepare a practical implementation plan. Safe mockups, workflow demonstrations, and storyboards are encouraged; no unapproved human testing is allowed.

Competition Experience

- At competition, teams demonstrate how the solution works, pitch the implementing organization, and respond to an adoption barrier such as budget reduction, staff resistance, privacy concern, or patient-access complication.

Required Work Product

- Solution brief; implementation roadmap; demonstration/prototype/workflow; budget; impact metrics; live defense.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Need and user definition	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Solution quality and usability	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Clinical/public-health value	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Implementation and operational feasibility	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Financial sustainability and metrics	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Demonstration and defense	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout Pitch Day entry does more than answer the prompt. It fully inhabits the role of the healthcare solution designer, performs the required action—**design**—and uses the signature mechanic, **demonstration, prototype, workflow, or storyboard requirement**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.

9. The Ledger

FINANCIAL | Solo-only | ON-SITE: STANDARD

Student role: Household financial planner **Primary action:** Plan **Signature mechanic:** Budget board with unexpected expense cards

Competitors manage a fictional individual or household facing healthcare-related financial choices involving income, insurance, deductibles, debt, emergency savings, treatment costs, risk, and competing obligations. The goal is a sustainable and humane plan, not simply the lowest short-term expense.

Preparation Before Competition

- Study budgeting, insurance fundamentals, debt, savings, risk, consumer choices, and healthcare affordability. The household scenario is released at the event.

Competition Experience

- Competitors create a budget and decision plan, then draw an unexpected event card—such as a job loss, emergency visit, coverage change, or medication cost increase—and revise their strategy.

Required Work Product

- Budget worksheet; coverage/care decision; risk and savings plan; revision rationale.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Calculation accuracy and case reading	30	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Financial decision quality	30	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Healthcare-access and human-impact reasoning	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Response to unexpected expense	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Clarity and completeness	5	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout The Ledger entry does more than answer the prompt. It fully inhabits the role of the household financial planner, performs the required action—**plan**—and uses the signature mechanic, **budget board with unexpected expense cards**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.

10. Market Call

FINANCIAL | Solo or team, 1–5 | ON-SITE: STANDARD

Student role: Investment desk analyst **Primary action:** Trade **Signature mechanic:** Live market-shock ticker

Competitors receive a fictional market mandate and newly released market/company information. They develop an investment view, allocation, or strategic position, name their catalysts and risks, and defend the call. The result is scored on reasoning at the time of decision, not hindsight performance.

Preparation Before Competition

- Study financial statements, valuation, risk/return, portfolios, market structure, macroeconomic signals, and healthcare-sector drivers where applicable. The case data are confidential until competition.

Competition Experience

- After timed analysis, competitors make a recorded call. A live market-shock ticker then releases a policy change, earnings surprise, competitor action, or macroeconomic event. Competitors must hold, hedge, exit, or revise—and explain why.

Required Work Product

- Investment thesis; allocation/position; risk plan; post-shock update; oral defense.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Data interpretation and analysis	30	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Thesis and strategic coherence	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Risk management and assumptions	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Response to market shock	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Communication and questioning	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout Market Call entry does more than answer the prompt. It fully inhabits the role of the investment desk analyst, performs the required action—**trade**—and uses the signature mechanic, **live market-shock ticker**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.
- Financial analysis is educational and simulation-based; it is not personal investment advice or a real solicitation of capital.

11. The Term Sheet

FINANCIAL | Team-only, 2–5 | PREPARATION-BASED

Student role: Deal team: investor, lender, or corporate finance adviser **Primary action:** Negotiate **Signature mechanic:** Live founder–investor/lender negotiation

Teams receive a company and financing context in advance. They recommend funding structure and terms based on valuation, capital needs, dilution, control, milestones, downside protection, and intended use of proceeds. The core event is negotiation, not spreadsheet display.

Preparation Before Competition

- Students receive the company dossier and rules before competition. Teams prepare a financing memorandum, draft term sheet, and negotiation strategy grounded in the company's facts and sector risks.

Competition Experience

- Teams first present their proposed deal. A judge role-playing founder, investor, lender, or board representative then negotiates valuation, control, milestones, covenant, liquidation, or timing points. A late performance update forces a revised offer or walk-away decision.

Required Work Product

- Investment memorandum; draft term sheet; financial assumptions; negotiation strategy; final deal rationale.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Financial and valuation logic	30	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Term quality and risk allocation	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Strategic use of proceeds	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Negotiation execution	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Adaptation and team command	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout The Term Sheet entry does more than answer the prompt. It fully inhabits the role of the deal team: investor, lender, or corporate finance adviser, performs the required action—**negotiate**—and uses the signature mechanic, **live founder–investor/lender negotiation**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.
- Financial analysis is educational and simulation-based; it is not personal investment advice or a real solicitation of capital.

12. Scarcity

FINANCIAL | Solo or team, 1–5 | ON-SITE: STANDARD

Student role: Healthcare resource-allocation analyst **Primary action:** Allocate **Signature mechanic:** Finite-budget token board

Competitors confront a health-system or policy scenario in which demand exceeds available funding, capacity, workforce, medication, or service access. They allocate a visibly limited set of resources and explain the economic incentives, patient consequences, and safeguards behind each choice.

Preparation Before Competition

- Study healthcare economics, supply/demand, insurance, incentives, market failures, regulation, externalities, and access. The specific allocation problem releases at competition.

Competition Experience

- Teams receive a finite budget/token board. Each allocation to one program visibly prevents allocation elsewhere. A mid-round change in demand, reimbursement, or supply requires reallocation and a transparent explanation of what must be delayed, protected, or redesigned.

Required Work Product

- Allocation board; economic analysis; access/equity safeguards; revised plan after shock.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Economic and incentive analysis	30	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Allocation quality and tradeoff transparency	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Access, equity, and patient outcomes	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Fiscal/operational feasibility	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Adaptation and oral defense	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout Scarcity entry does more than answer the prompt. It fully inhabits the role of the healthcare resource-allocation analyst, performs the required action—**allocate**—and uses the signature mechanic, **finite-budget token board**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.

13. Operating Margin

FINANCIAL | Team-only, 2–5 | PREPARATION-BASED

Student role: Service-line or clinic optimization team **Primary action:** Optimize **Signature mechanic:** Operations dashboard and constraint round

Teams receive a defined hospital or clinic operational problem in advance—such as bottlenecks, staffing, denied claims, revenue leakage, avoidable utilization, quality costs, or service-line pressure. Unlike The Turnaround, this event targets one operating system and requires a measurable improvement plan.

Preparation Before Competition

- The organization packet, metrics, and decision question are released before competition. Teams build a focused plan supported by data, financial assumptions, workflow redesign, quality safeguards, and implementation metrics.

Competition Experience

- Teams present to an operations committee using a dashboard. Judges then impose a constraint: fewer staff, a budget cut, a payer rule, a capacity limit, or a quality threshold. Teams must protect the core plan or deliberately redesign it.

Required Work Product

- Problem diagnosis; dashboard; operating/financial plan; implementation timeline; quality/access safeguards; revised constraint response.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Data-based problem diagnosis	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Financial sustainability	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Patient quality and access protection	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Operational feasibility	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Constraint response and defense	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout Operating Margin entry does more than answer the prompt. It fully inhabits the role of the service-line or clinic optimization team, performs the required action—**optimize**—and uses the signature mechanic, **operations dashboard and constraint round**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.

14. The Pipeline

FINANCIAL | Solo or team, 1–5 | PREPARATION-BASED

Student role: Biotech/pharma due-diligence committee **Primary action:** Diligence **Signature mechanic:** Investment committee with late clinical or patent update

Competitors assess whether a fictional biotechnology or pharmaceutical opportunity can succeed scientifically and commercially. They examine clinical evidence, trial risk, patent life, manufacturing, regulation, pricing, reimbursement, competition, capital needs, and launch strategy.

Preparation Before Competition

- The company dossier and therapeutic domain release before competition. Competitors research the area, complete a diligence analysis, and prepare a recommendation with scenarios and key assumptions.

Competition Experience

- Teams deliver an investment-committee recommendation. During questioning, they receive a new trial endpoint result, patent challenge, reimbursement decision, manufacturing issue, or competitor entry that forces a funding, valuation, partnership, or launch revision.

Required Work Product

- Diligence report; clinical-to-commercial assessment; risk matrix; scenario model; recommendation and live revision.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Clinical/scientific interpretation	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Commercial/regulatory/reimbursement analysis	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Financial logic and risk modeling	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Strategic recommendation	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Response to late update	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout The Pipeline entry does more than answer the prompt. It fully inhabits the role of the biotech/pharma due-diligence committee, performs the required action—**diligence**—and uses the signature mechanic, **investment committee with late clinical or patent update**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.
- Financial analysis is educational and simulation-based; it is not personal investment advice or a real solicitation of capital.

15. Claim Denied

FINANCIAL | Solo or team, 1–5 | ON-SITE: STANDARD

Student role: Claims appeal and adjudication panel **Primary action:** Adjudicate **Signature mechanic:** Two-chair payer–patient hearing

Competitors receive a fictional denied claim, policy language, medical record, coding/billing context, and appeal facts. They must accurately articulate both the patient-access case and payer-administration rationale before issuing a fair, evidence-based resolution.

Preparation Before Competition

- Study coverage design, reimbursement, medical-necessity review, appeals, documentation, coding context, patient access, and ethical decision-making. The case is released on-site.

Competition Experience

- Competitors first prepare separate patient and payer analyses. At the hearing, two judges represent those perspectives and challenge the proposed resolution. A late document or policy clarification may alter the outcome.

Required Work Product

- Claim analysis; patient/payer comparison; adjudication decision; appeals/process improvement recommendation.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Case and coverage interpretation	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Clinical evidence and appropriateness	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Fairness and access reasoning	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Financial/administrative defensibility	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Hearing performance and resolution clarity	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout Claim Denied entry does more than answer the prompt. It fully inhabits the role of the claims appeal and adjudication panel, performs the required action—**adjudicate**—and uses the signature mechanic, **two-chair payer–patient hearing**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.

16. The Turnaround

SYSTEMS | Team-only, 2–5 | PREPARATION-BASED

Student role: Executive leadership team **Primary action:** Rescue **Signature mechanic:** Multi-round boardroom rescue simulation

Teams take over a failing healthcare organization facing interlocking problems in finance, staffing, quality, access, governance, and public confidence. This is an enterprise rescue—not a single workflow optimization. Teams must choose a strategic direction and protect essential care while restoring viability.

Preparation Before Competition

- The organization dossier and turnaround mandate are released before competition. Teams prepare a board-ready stabilization and multi-year recovery plan with financial, workforce, quality, communications, and governance components.

Competition Experience

- The boardroom simulation has multiple rounds: initial strategy, hostile board questions, a new crisis such as a payer shift or safety incident, and a final recovery/accountability briefing. Teams must show what they will change, what they will protect, and what they will stop doing.

Required Work Product

- Root-cause diagnosis; stabilization plan; multi-year strategy; financial/workforce/quality plan; risk register; updated response.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Strategic diagnosis and coherence	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Quality, safety, and access protection	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Financial and operational resilience	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Workforce and governance plan	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Implementation and risk management	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Live executive adaptation	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout The Turnaround entry does more than answer the prompt. It fully inhabits the role of the executive leadership team, performs the required action—**rescue**—and uses the signature mechanic, **multi-round boardroom rescue simulation**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.

17. Signal vs. Noise

SYSTEMS | Solo or team, 1–5 | ON-SITE: STANDARD

Student role: Clinical technology review board **Primary action:** Validate **Signature mechanic:** Technology approval gate and incident drill

Competitors assess a fictional AI or digital-health product for true clinical, operational, and financial value. They distinguish good marketing from credible evidence by reviewing validation data, subgroup performance, workflow fit, privacy, cybersecurity, cost, adoption, and governance.

Preparation Before Competition

- Study digital-health evaluation, AI limitations, clinical evidence, bias, privacy, interoperability, cybersecurity, workflows, and business models. The product dossier is released at competition.

Competition Experience

- Teams run a formal approval gate: approve, pilot, restrict, redesign, reject, or defer. They must define conditions and monitoring thresholds. Then an incident drill reveals a false negative, subgroup disparity, outage, data-sharing concern, or clinician work-around.

Required Work Product

- Technology decision; evidence audit; deployment/oversight conditions; incident response; financial/operational assessment.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Clinical value and patient safety	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Data/evidence and limitation analysis	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Equity, privacy, and governance	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Operational and financial feasibility	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Approval conditions and incident response	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout Signal vs. Noise entry does more than answer the prompt. It fully inhabits the role of the clinical technology review board, performs the required action—**validate**—and uses the signature mechanic, **technology approval gate and incident drill**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.

18. Seed Round

SYSTEMS | Solo or team, 1–5 | PREPARATION-BASED

Student role: Healthcare startup founding team **Primary action:** Found **Signature mechanic:** Investor due-diligence gauntlet

Competitors build an independent healthcare venture with a defined customer, unmet need, business model, revenue logic, funding ask, milestones, and responsible-growth plan. Unlike Pitch Day, Seed Round asks whether a new company deserves capital and can become sustainable.

Preparation Before Competition

- The event prompt and submission rules release in advance. Teams validate the problem, develop the company model, build financial assumptions, identify competitors and risks, and submit venture materials before competition.

Competition Experience

- Teams pitch investors, then face a due-diligence gauntlet on market size, price, unit economics, clinical evidence, privacy, regulation, competition, and funding use. Judges may challenge one core assumption, requiring a revised path or a reasoned defense.

Required Work Product

- Executive summary; pitch deck; financial model; funding ask/use of funds; milestones; risk and impact plan.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Problem-market fit and validation	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Business model and financial logic	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Innovation and competitive strategy	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Responsible feasibility and growth	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Funding strategy and milestones	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Investor defense	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout Seed Round entry does more than answer the prompt. It fully inhabits the role of the healthcare startup founding team, performs the required action—**found**—and uses the signature mechanic, **investor due-diligence gauntlet**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.
- Financial analysis is educational and simulation-based; it is not personal investment advice or a real solicitation of capital.

19. Borders & Budgets

SYSTEMS | Solo or team, 1–5 | PREPARATION-BASED

Student role: Contextual health-implementation team **Primary action:** Implement **Signature mechanic:** Geographic implementation board and logistics shock

Competitors receive a specified country or region and health challenge in advance. They design a response grounded in disease burden, local infrastructure, workforce, culture, supply chains, governance, financing, affordability, and community partnership. The central test is implementability in context.

Preparation Before Competition

- The setting, health challenge, baseline data, and rules are released before competition. Teams research responsibly, declare assumptions, develop a budget and phased implementation plan, and submit materials.

Competition Experience

- Teams present at an implementation board. A logistics shock—weather, supply interruption, funding cut, policy change, workforce departure, or changing disease burden—requires teams to protect core services and revise sequencing.

Required Work Product

- Context analysis; fixed-budget allocation; partnership and implementation roadmap; metrics; sustainability plan; live revision.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Contextual accuracy and needs analysis	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Clinical/public-health impact	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Budget logic and allocation	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Local feasibility and sustainability	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Equity, ethics, and risk response	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Communication	5	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout Borders & Budgets entry does more than answer the prompt. It fully inhabits the role of the contextual health-implementation team, performs the required action—**implement**—and uses the signature mechanic, **geographic implementation board and logistics shock**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
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20. On Record

SYSTEMS | Team-only, 2–5 | PREPARATION-BASED

Student role: Health crisis communications team **Primary action:** Communicate **Signature mechanic:** Live press room and misinformation escalation

Teams prepare an evidence-based response to a fictional health misinformation, patient-trust, product-safety, outbreak, privacy, or organizational-confidence crisis. They must communicate accurately and humanely while aligning messages with an actual operational response. The event rewards trustworthy action, not public-relations polish alone.

Preparation Before Competition

- The initial scenario and submission rules release in advance. Teams create a communications strategy, stakeholder map, message architecture, selected materials, and escalation plan before competition.

Competition Experience

- At competition, teams conduct a press briefing and stakeholder briefing. Judges act as reporters, patients, staff, advocates, and board members. A live rumor, data correction, video clip, or affected-community concern tests whether the team can correct course without losing credibility.

Required Work Product

- Audience map; core messages; channel/timing strategy; misinformation protocol; selected materials; live briefing and update.

Event Rubric — 100 Points

Criterion	Points	Judge standard
Accuracy, evidence, and transparency	25	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Audience strategy and accessibility	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Operational integration and feasibility	20	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Trust, ethics, and equity	15	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Adaptation to escalation	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.
Team communication	10	Specific, evidence-based, feasible, and responsive to the event's unique role, live mechanic, and case constraints.

Student Success Standard

A standout On Record entry does more than answer the prompt. It fully inhabits the role of the health crisis communications team, performs the required action—**communicate**—and uses the signature mechanic, **live press room and misinformation escalation**, to prove that its reasoning holds up when conditions change.

Rules and Boundaries

- Use only authorized materials and follow all monitored-preparation instructions.
- Ground all claims in the event packet or clearly labeled assumptions.
- Every team member must understand the full recommendation and may be questioned individually.
- The annual event guide may specify exact submission deadlines, room procedures, equipment, and permitted reference materials.